

CITYCARE GENERAL HOSPITAL

# PATIENT ADMISSION STANDARD OPERATING PROCEDURE

Inpatient Admissions — Elective, Emergency & Transfer

|                   |                                              |
|-------------------|----------------------------------------------|
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## 1. PURPOSE & SCOPE

This SOP governs the end-to-end admission process for all patients entering CityHospital as inpatients, whether elective, emergency, or transfer cases. It ensures a safe, efficient, and patient-centred admission experience while meeting regulatory documentation standards.

- Standardise admission procedures across all entry points to the hospital.
- Ensure timely clinical assessment and appropriate bed allocation for all patients.
- Protect patient rights and obtain valid informed consent before care commences.
- Maintain accurate, complete, and legally compliant admission documentation.
- Support seamless handover between admissions staff and ward clinical teams.

2. DEFINITIONS & ABBREVIATIONS

| Term                     | Definition                                                                           |
|--------------------------|--------------------------------------------------------------------------------------|
| Elective Admission       | Pre-planned admission scheduled in advance by a referring physician.                 |
| Emergency Admission      | Unplanned admission through the Emergency Department or direct referral.             |
| Transfer Admission       | Patient arriving from another healthcare facility for ongoing care.                  |
| Pre-Admission Assessment | Clinical and administrative evaluation completed before the admission date.          |
| MRN                      | Medical Record Number — unique patient identifier assigned at first registration.    |
| Bed Management           | Coordinated process of allocating, tracking, and optimising inpatient bed occupancy. |
| Admitting Physician      | Registered specialist responsible for the patient's inpatient care plan.             |
| Triage                   | Clinical prioritisation process assigning urgency level to incoming patients.        |
| SBAR                     | Situation–Background–Assessment–Recommendation communication framework.              |

3. ROLES & RESPONSIBILITIES

| Role                | Department             | Responsibilities                                                 |
|---------------------|------------------------|------------------------------------------------------------------|
| Admissions Clerk    | Admissions Office      | Registration, documentation, insurance verification, bed request |
| Admitting Physician | Relevant Specialty     | Clinical orders, admission note, treatment consent               |
| Triage Nurse        | Emergency / Admissions | Initial clinical assessment, prioritisation, vital signs         |
| Bed Manager         | Bed Management         | Real-time bed allocation, ward coordination, transfer oversight  |
| Ward Nurse          | Assigned Ward          | Receive patient, admission assessment, care plan initiation      |
| Social Worker       | Social Services        | Financial, social, and safeguarding assessments as needed        |
| Billing Team        | Finance                | Insurance pre-authorisation, patient financial counselling       |

## 4. PRE-ADMISSION PLANNING

Effective pre-admission planning minimises delays, reduces unnecessary investigations, and improves patient experience. This applies to all elective admissions scheduled 48 hours or more in advance.

### 4.1 Pre-Admission Checklist

- Confirm surgical/procedure date and admitting physician availability.
- Send pre-admission questionnaire to patient covering medical history, medications, and allergies.
- Arrange pre-operative investigations: bloods, ECG, imaging as per specialty protocol.
- Complete insurance pre-authorisation; counsel patient on out-of-pocket costs.
- Confirm nil-by-mouth instructions and any pre-operative medication adjustments.
- Conduct anaesthesia pre-assessment for surgical patients (at least 24 hours before admission).
- Arrange interpreter services, mobility aids, or special dietary needs if identified.

### 4.2 Patient Communication

- Patient receives written or digital admission instruction letter at least 48 hours before admission.
- Admission date, time, ward location, items to bring, and prohibited items clearly stated.
- Contact number for queries provided; dedicated pre-admission helpline available 08:00–20:00.

## 5. ELECTIVE ADMISSION WORKFLOW

- Step 1.** Patient arrives at Admissions Reception; photo ID and insurance card presented and verified.
- Step 2.** Admissions clerk retrieves or creates patient MRN; demographic data confirmed and updated in EMR.
- Step 3.** Financial counsellor completes billing briefing; patient or guardian signs financial consent.
- Step 4.** Admissions nurse performs preliminary assessment: vital signs, weight, height, medication reconciliation, allergy verification.
- Step 5.** Admissions clerk notifies bed manager of pending admission; preferred ward or room communicated.
- Step 6.** Bed manager allocates appropriate bed; ward nurse informed of incoming patient ETA.
- Step 7.** Porter transports patient to assigned ward with admission notes and property list.
- Step 8.** Ward nurse completes full admission nursing assessment within 2 hours; care plan initiated.
- Step 9.** Admitting physician reviews patient within 4 hours; admission orders entered in EMR.
- Step 10.** Patient and family oriented to ward environment, call bell, visiting hours; patient rights booklet distributed and acknowledged.

**NOTE:** Target time from patient arrival to bed allocation: less than or equal to 60 minutes for elective admissions.

## 6. EMERGENCY ADMISSION WORKFLOW

Emergency admissions bypass standard elective procedures. Patient safety and speed of clinical intervention take absolute priority over administrative processes.

**Step 1.** Patient arrives at Emergency Department; triage nurse assigns priority level (1–5) within 5 minutes of arrival.

**Step 2.** Resuscitation commenced if Level 1; stabilisation bay activated; crash team notified.

**Step 3.** Emergency physician assesses and documents presenting complaint, history, and examination findings.

**Step 4.** Investigations ordered stat; critical results communicated to physician immediately.

**Step 5.** Decision to admit made by emergency physician; specialist consult requested as needed.

**Step 6.** Admissions office notified; MRN assigned or retrieved; patient identity confirmed via wristband.

**Step 7.** Bed manager activates emergency bed request; overflow protocol applied if bed occupancy is critical.

**Step 8.** Patient transferred to ward or ICU with full verbal SBAR and written handover; transfer checklist completed.

**Step 9.** Relatives or next of kin notified as soon as the patient is stable and identified.

**Step 10.** Post-admission administrative documentation completed by admissions clerk within 2 hours.

**WARNING: Level 1 (RED) patients must be seen by a physician within 0 minutes of arrival. No administrative process may delay immediate life-saving care.**



## 7. INTER-HOSPITAL TRANSFER ADMISSION

- Receiving physician confirms acceptance of transfer in writing or via EMR before transfer commences.
- Transferring hospital provides complete clinical summary, investigation results, and current medication chart.
- Bed manager confirms receiving ward and notifies nursing team at least 30 minutes before expected arrival.
- Transfer ambulance crew provides verbal SBAR handover to receiving nurse on arrival.
- Receiving nurse verifies patient identity, checks all accompanying documentation, and initiates admission assessment.
- Transferring facility documentation scanned and linked to patient MRN in EMR within 2 hours of arrival.
- Any clinical deterioration during transfer must be documented and communicated to the receiving physician immediately.

## 8. PATIENT ASSESSMENT ON ADMISSION

### 8.1 Nursing Admission Assessment (within 2 hours)

- Vital signs: BP, HR, RR, SpO2, temperature, pain score (0–10 NRS).
- Falls risk assessment (Morse Fall Scale); pressure injury risk (Braden Scale).
- Nutritional screening (MUST tool); malnutrition action plan if score is 2 or higher.
- Cognitive and mental health screening (abbreviated mental test score).
- Skin integrity assessment; document and photograph any pre-existing wounds or marks.
- Social history: living situation, carer availability, safeguarding concerns.
- Medication reconciliation: full current medication list verified and entered in EMR.

## 9. BED MANAGEMENT & ALLOCATION

- Real-time bed occupancy dashboard monitored by bed manager continuously, 24 hours/7 days.
- Bed allocation follows clinical priority: ICU > HDU > specialty ward > general ward.
- Gender-specific wards maintained; mixed-gender placement only in clinical emergency with patient or guardian consent documented.
- Isolation rooms prioritised for patients with confirmed or suspected infectious conditions.
- Bed turnaround target: cleaned and ready within 45 minutes of previous patient discharge.
- Escalation protocol activated when bed occupancy exceeds 90%: elective admissions reviewed, early discharge promoted, ward managers alerted.
- Overflow protocol: day procedures unit and elective surgery wards designated as overflow capacity zones.

## 10. DOCUMENTATION, EXCEPTIONS & BEST PRACTICES

### 10.1 Required Admission Documents

- Patient registration form, photo ID copy, insurance card copy.
- Signed financial consent and privacy notice.
- Physician admission note and initial orders in EMR.
- Nursing admission assessment form including falls and pressure risk scores.
- Medication reconciliation form signed by pharmacist and nurse.
- Patient rights and responsibilities booklet — acknowledged by patient signature.

### 10.2 Exceptions

- Unidentified (unconscious/unknown) patients: assign temporary MRN 'JOHN/JANE DOE + date+time'; ID protocol activated.
- Paediatric admissions: parent or guardian consent is mandatory; child life specialist notified for admissions exceeding 48 hours.
- Patients refusing admission: document informed refusal; physician counselling attempted; refusal form signed by patient.

### 10.3 Best Practices

- Use AIDET communication framework (Acknowledge, Introduce, Duration, Explanation, Thank You) during admission.
- Confirm patient's preferred name and language at registration; activate interpreter service proactively if needed.
- Conduct admission medication reconciliation as a collaborative pharmacist-nurse process for all high-risk medications.
- Photograph all pre-existing wounds, bruises, or skin conditions with the patient's written consent.